



### Acute Gastroenteritis/Dehydration Pathway

#### Inclusion Criteria

- Presumed gastroenteritis
- Age >6 months; and up to 18 years
- Weight >8kg
- Vomiting and/or diarrhea of recent onset; not due to chronic disease or suspected other condition; with or without fever, nausea, or abdominal pain

#### Exclusion Criteria

- Toxic or ill appearance
- Altered mental status
- Severe dehydration, hypotensive, hyponatremia (<130 mEq/L) or hypernatremia (>155 mEq/L)
- Blood in stool or vomit
- Chronic or comorbid conditions (medically complex children, renal failure, cardiac disease, Hirschsprung's, diabetes)
- History of abdominal surgery; or intussusception
- On diuretics
- Possible surgical abdomen (bilious emesis; signs or symptoms of bowel obstruction)

### **Virtual Medical Care**

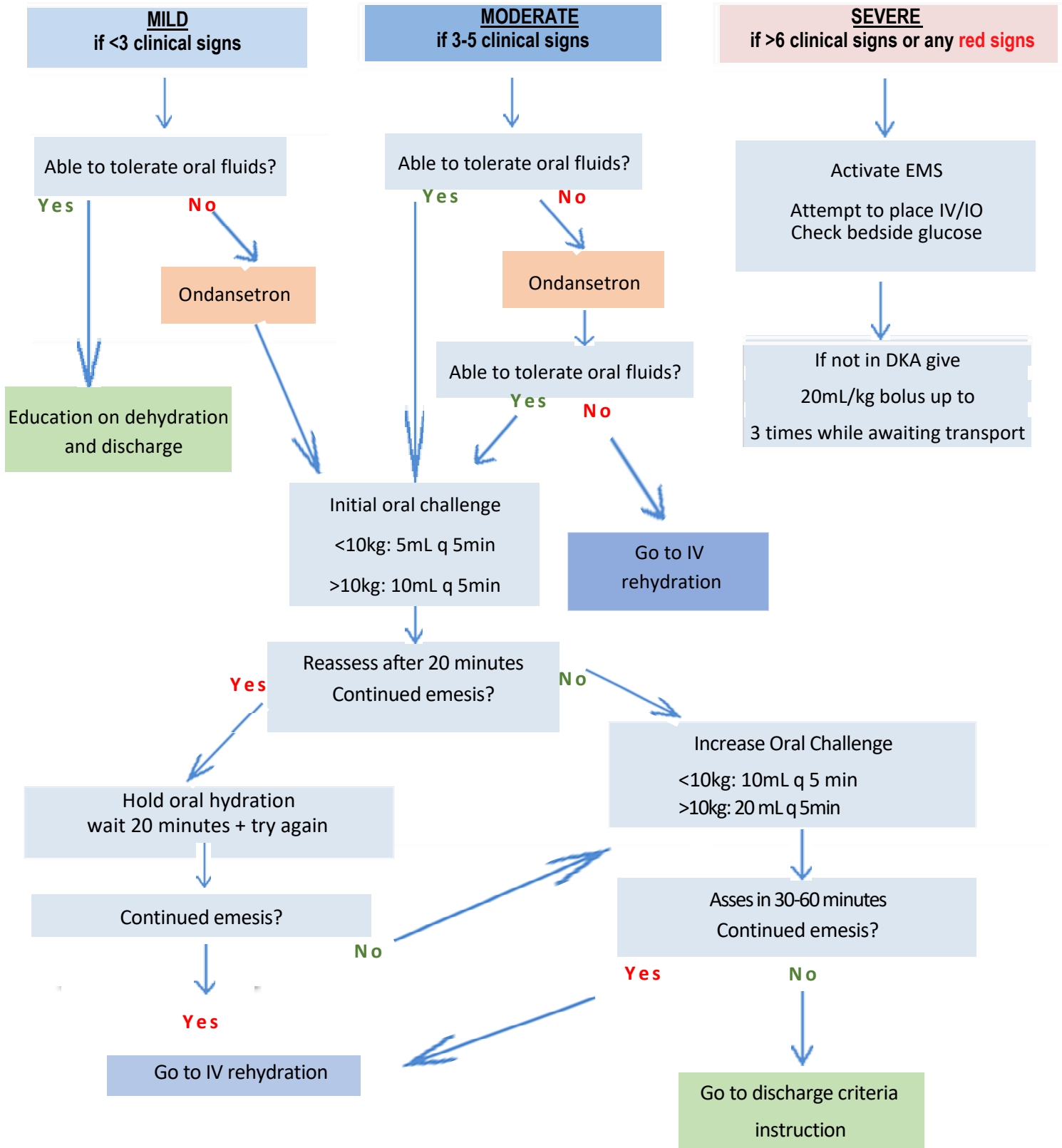
#### **Indications for in-person evaluation for chief complaint vomiting and/or diarrhea:**

- <6 months
- <8kg
- Visible blood in the stool or vomit
- Bilious emesis
- Concerning history (nonclassical symptoms, polyuria, polydipsia)
- Prolonged duration of illness (emesis >72 hours, diarrhea >7 days, fever >5 days)
- Frequent and substantial volumes of diarrhea (>8/day)
- Not able to tolerate oral fluids (ie G-tube, Nasogastric NG tube)
- Caregiver unable to successfully administer oral rehydration (sips of fluids throughout the day)
- Parent description of ill-appearance
- Severe abdominal pain, signs of peritonitis (not able to walk or jump up and down)
- Signs of dehydration
  - Low/decreased UOP (no UOP in last 8 hours)
  - Dry or cracked lips
  - Not producing tears
  - Not ambulating; and/or decreased activity/listless/irritability
- If scheduling in person visit, please do so at a site which has IV fluid capabilities; this may mean pediatric clinic, urgent care, or ER depending on the resources currently available

## Signs and Symptoms of Dehydration

- Activity decreased
- Capillary refill >2 sec
- Decreased or no tears present
- Dry mucus membranes

- Sunken eyes
- Respiration abnormal or significantly increased
- Abnormal pulses / perfusion
- Decreased skin turgor / recoil



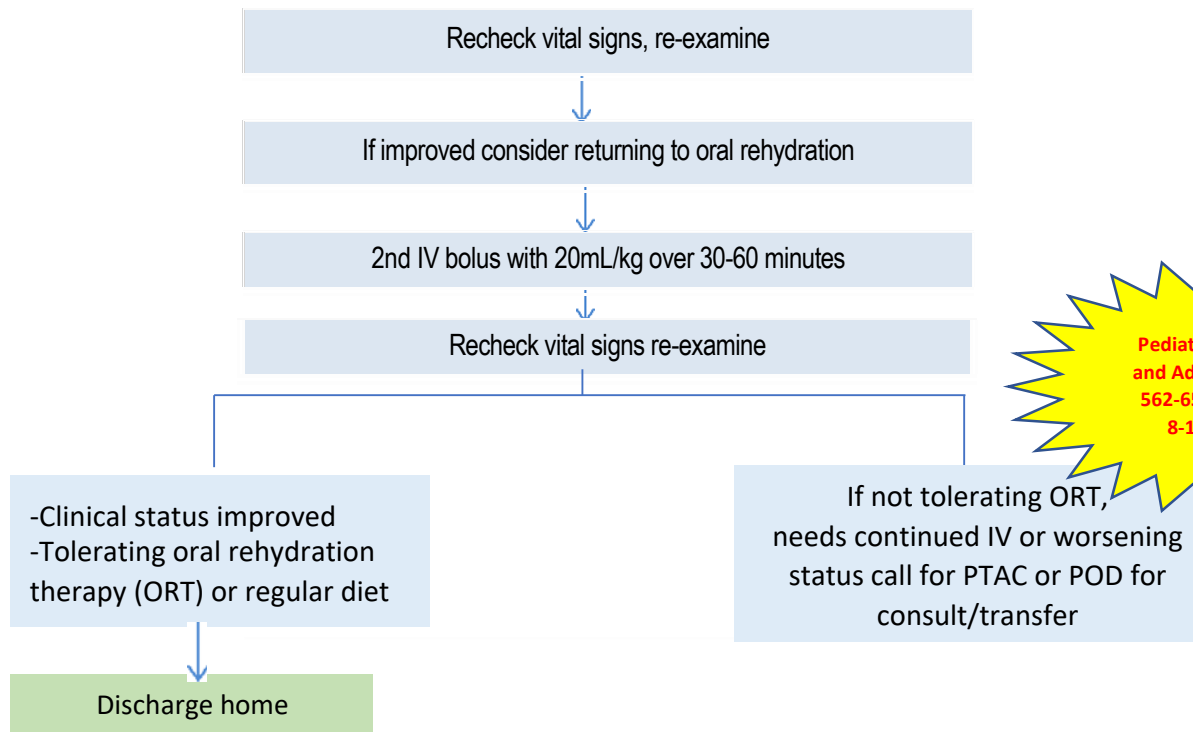
## IV Rehydration

### 1st IV bolus

- NS 20mL/kg over 30-60 minutes; max dose=1000ml (10ml/kg if any concern for cardiac dysfunction)
- Upon initiation of IV, check bedside blood sugar; Go to "hypoglycemia" if needed
- Consider electrolytes, depending on degree of dehydration
- Consider ondansetron if not already given or tolerated, may be give PO or IV

Note: may give bolus rapidly if signs of shock or sepsis

Note: consider myocarditis if patient deteriorates rapidly after fluid boluses



**Pediatric Transfer  
and Advice Center:  
562-658-3980 OR  
8-137-3980**

### Discharge instruction:

- Continue ORT for 4-6 hours at home then resume regular diet. (10 mL/kg for each episode of emesis or diarrhea)
- Use lactose free formula if primarily formula fed
- Consider lactobacillus for 5 days? (although recent studies show no efficacy)
- Follow up established
- Family education
- Imodium AD not recommended in younger children
- Bismuth subsalicylate not recommended in younger children
- Zinc supplementation helpful only in developing countries

**Zofran dosing- please also refer to Lexicomp (caution with dosing and be aware of potential for Prolonged QT – especially in doses >16mg)**

IV: Infants and Children  $\geq 1$  month: 0.15 or 0.3 mg/kg/dose once; maximum dose: 8 mg/dose

Oral: Infants and Children 6 months to 10 years,  $\geq 8$  kg (Freedman 2006):

8 to 15 kg: 2 mg/dose once

>15 to 30 kg: 4 mg/dose once

>30 kg: 8 mg/dose once

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**Approved:** SCAL Regional Pediatric COS, Regional COS UCC, Regional Pediatric Hospital Medicine PIC Groups: **June 2024**

**References:**

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